

Examining Researcher Experiences and Tensions Around Participant Engagement in Health HCI Research

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Abstract

Research success in Health Human-Computer Interaction (Health HCI) often hinges on effective participant engagement. However, the definition and measurement of engagement can vary significantly depending on the context of the specific study. This variation necessitates using different methods and strategies to address issues related to low participation. This paper explores the perceptions of Health HCI researchers regarding the challenges they encounter in facilitating participant engagement and achieving meaningful research impact. Through semi-structured interviews with junior and more senior researchers, we examine insights into their experiences and the barriers they encounter. Our findings highlight a relationship between intended research contributions and researcher focus on engagement, a distinction between probe and product mindsets in Health HCI research, and complexities surrounding researcher well-being as they strive for compelling health research contributions. We also discuss implications for individual researchers, institutions, and the broader field of Health HCI, aiming to enhance the overall quality and relevance of research outcomes.

CCS Concepts

• **Human-centered computing** → HCI theory, concepts and models.

Keywords

Health Human-Computer Interaction Research, Participant Engagement, Researcher Challenges

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1 Introduction

Participant engagement plays a pivotal role in determining the success of Human-Computer Interaction research in Health (i.e., Health HCI). As researchers aim to maximize their impact and outcomes, it is essential to understand the barriers that impede participant engagement thoroughly. Low engagement not only hampers data collection, but can also lead to the premature termination of a study or compromise the validity and reliability of findings. Identifying and addressing these barriers is crucial for researchers in fostering a collaborative environment that encourages active participation and yields more robust results.

This paper explores how Health HCI researchers perceive the multifaceted challenges of enhancing participant engagement in their studies. We examine the potential overlooking of participant experiences and benefits. This overlooking may stem from an intense focus on generating valuable contributions to the field, potentially at the expense of understanding participant nuanced needs and preferences. We address potential challenges of researcher well-being. By analyzing these interconnected factors, we aim to illuminate how these dynamics influence the effectiveness of research in fostering genuine participant involvement.

We examine these issues through semi-structured interviews with Health HCI researchers, including junior scholars and more senior researchers, to gain insight into their experiences, challenges, and perspectives on participant engagement. Our findings contribute to understanding how intended contributions influence researcher focus on participant engagement, the probe versus product mindset in Health HCI, and the complexities of researcher well-being in pursuing important and impactful health research. We then discuss these findings regarding considerations for individual researchers, institutions, and the field on changing current methods and research practices that may enable researchers to enhance participant engagement.

2 Related Work

Engagement refers to a person's level of interaction with an intervention or tool. Research in HCI uses diverse definitions of engagement, drawing on diverse theoretical frameworks [4], resulting in varied methods and metrics, such as login frequency, completion of modules, and page views [7]. When a person fails to engage or disengages after an initial interaction, the potential benefits of an intervention cannot be fully realized [9]. However, engagement alone does not ensure success. People must perceive value and find relevant tools that effectively support their health

goals. Investigating barriers to engagement is thus a key component of strategies for enhancing well-being through technology.

Health technologies often go unused [7, 10, 18]. Challenges arise amidst external factors or due to the design of the intervention itself, making it essential to examine disengagement further. Disengagement might refer to not even beginning to use a tool, to momentary lapses in attention, or a complete abandonment of a tool or service [12]. Optimal engagement may depend on individual attributes or context, the mechanisms of an intervention, and desired outcomes [10]. Perpetual, consistent engagement should not be assumed to be optimal, as pauses or even moving on from technology can be advantageous [8]. That said, understanding engagement to improve it is often a goal in Health HCI research [4]. While quantitative data obtained through logs can offer a detailed and unobtrusive assessment of interaction with a technology, complementary qualitative methods are vital for fully understanding and interpreting a range of experiences [10].

More participatory approaches to developing and evaluating digital behavior change interventions can enhance engagement and their efficacy [10]. For example, Unertl et al. examined Community-Based Participatory Research (CBPR) as an approach to improve health outcomes for under-served populations. They examined five health informatics initiatives that used CBPR and found that although CBPR effectively promoted community engagement, it can also come with challenges, such as sustainable partnerships, cultural misalignments, and technology-related issues [17].

Researchers designing Health HCI interventions may be uncertain about addressing disengagement, especially in emotionally demanding research areas where their well-being poses additional challenges [5, 11, 20]. The HCI community is currently discussing how to support researchers better, emphasizing the necessity for teams and collaborators to support one another during emotionally demanding research and for institutes and corporations to provide essential resources like trauma-informed and well-being training to mitigate harm to researchers [5]. By taking a researcher-centered view of engagement in this study, we sought to contribute to our understanding of opportunities to support researchers working in Health and HCI, which could result in better experiences for study participants and facilitate better and more impactful research.

3 Methods

To examine how Health HCI researchers perceive challenges in pursuing participant engagement and positive impact, we interviewed self-identified Health HCI researchers who conducted research in treatment, screening, diagnosis, health monitoring, or intervention through studies that: (1) collected data from participants over multiple days, (2) involved collaboration between participants and healthcare providers, and (3) utilized patient-generated health data. The study protocol was reviewed by our university's Institutional Review Board.

A total of 14 Health HCI researchers participated in interviews: 7 pre-terminal degree “junior” and 7 post-terminal degree “senior” researchers. The first author recruited participants through professional connections, including a Health HCI workshop at

Table 1: Overview of Participants. All self-identified as Health HCI researchers.

Paper ID	Researcher Experience Level	Research Title
JR-P1	Student	PhD Student
JR-P2	Student	PhD Student
JR-P3	Student	PhD Student
JR-P4	Student	PhD Student
JR-P5	Student	PhD Student
JR-P6	Student	PhD Student
JR-P7	Student	PhD Student
SR-P1	Junior to Mid-Career Faculty	Faculty
SR-P2	Senior Investigator	Executive Director
SR-P3	Junior to Mid-Career Faculty	Assistant Professor
SR-P4	Junior to Mid-Career Faculty	Honorary Associate Professor
SR-P5	Junior to Mid-Career Faculty	Associate Professor
SR-P6	Junior to Mid-Career Faculty	Faculty
SR-P7	Senior Investigator	Senior Investigator, Family Physician, Professor, Clinical Professor

CHI 2024 and local health-related professional networking events. We conducted 30 to 60-minute semi-structured interviews via Zoom, exploring their prior research experiences, challenges with participant engagement, and associated strategies.

The first author's experiences in Health HCI research informed these interviews, including an initial focus on questions the research community might have for participants who withdrew from studies and potential researcher approaches to increasing participant engagement. Initial interviews found that early-career researchers encountered similar challenges, including limited project focus on improving participant engagement. Consequently, we expanded the focus of interviews to the obstacles hindering further exploration of participant engagement. Senior researchers shared insights from their careers and perspectives on junior researcher findings. All interviews were audio recorded, transcribed using Otter.ai, and coded using Atlas.ti. The first author led the initial coding, and the team collaborated to develop and refine themes through memoing, affinity diagramming, and storyboarding.

4 Findings

Researcher participants described how the expected contributions for a given project or field can shape whether and how much they focus on participant engagement in their research. One prominent example is whether researchers saw the systems, services, or artifacts they were designing as *probes*, intended to contribute to knowledge, as a *prototypes* on which they would iterate as they advance toward products. Researcher participants were often

conflicted over how much to focus on understanding and addressing engagement, particularly of individual study participants, and they described this as challenging. In the following sections, we elaborate on each of these themes.

4.1 Intended Contribution Affects Researcher Focus on Participant Engagement

The intended contribution of research shapes its design, methods, conduct, and reporting [19]. SR-P1, SR-P2, and SR-P6 shared their perspective in the context of academic research goals, saying academic researchers are “*trying to contribute to fundamental research understanding*” (SR-P6) and hope future researchers can “*build something from their findings*” (JR-P3). However, they noted these objectives are also often overshadowed by the “*publish or perish*” model prevalent in academia (SR-P1, SR-P2, SR-P6). In contrast, SR-P2 observed that industry researchers, such as UX researchers, rarely publish their work but instead focus on the product and user experience. They felt industry researchers typically concentrate on “*doing the work*” and may share insights through “*a blog post*,” with their primary goal being to inform the product development process.

Researchers described how differences in goals could affect their motivation to investigate and address engagement. Academic researchers might prioritize publication and have less incentive to explore low engagement or to enhance participant experience unless findings align with a researcher’s anticipated outcome expectations. Researchers may find that taking time to understand or address participant engagement may lead to challenges due to resource limitations, which are worth the effort when they have sufficient data for publishing. SR-P6 pointed out that “*academia looks for things to be clean... with participants neatly fitting within your inclusion criteria and not having other factors in their lives—like comorbidities—that could affect the results so that you can isolate [a] factor*.” Both SR-P6 and JR-P1 described experiences in projects where achieving “clean” data, often by excluding outliers, was seen as essential for obtaining meaningful and clear findings. However, this emphasis on “clean” data limits exploring issues experienced by participants who do not fit such stringent standards.

4.1.1 Probe versus Product Mindset. Several researchers emphasized distinctions between probe-based and product-oriented approaches. Although researchers often used the term “prototype,” they described approaches using probes as tools for learning, wherein participant benefits may be considered optional. SR-P1 claimed that researchers “*are often creating [probes]. So our goal is ... to use this for a few weeks and evaluate [the] approach... then we’re going to throw [it] away*.” SR-P1 added that “*a good [probe], has good usability*” and a high-quality probe could help “*participants maintain some interest in using [the probe] for a little bit longer*,” but their motivation for such refinement remained the learning associated with a probe. In other words, if the lessons learned from a lack of engagement were informative for the field and future work by others, then the project was a success despite limited engagement, or perhaps even because of it.

In contrast, some researchers framed projects more as prototypes or research products, emphasizing participant expectations of benefits. For example, JR-P3 commented that “*if [a researcher]*

can figure out a way to balance this triangle [between] participant motivation, researcher motivation, and [research] goals ... you can get pretty good [research and health] outcomes.” This aligns with SR-P2, who believes “*the balance between efficacy¹ and effectiveness² research is salient*” and that researchers should “*focus on what works in the real world ... not how you would optimally like [participants] to engage*”. JR-P7 expressed concern over participant dropouts, wondering if they were “*designing studies in ways that are off-putting to folks*”, and acknowledged their focus was on ethical aspects, not strategies to keep participants engaged.

Researchers expressed curiosity about engagement, but few planned to investigate it further, with some studies prioritizing naturalistic observation over engagement strategies. SR-P3 noted that explicit engagement tactics might bias participants, making it challenging to balance data quality and participant retention. SR-P3 added, “*engagement is hard, [trying] to figure out how to balance getting useful data versus keeping [a few participants] from dropping out*”. SR-P6 felt it was not the researcher’s role to determine beneficial aspects for participants but to focus on “*how [researchers can] contribute understanding back to the research field rather than how [researchers] benefit the individual participants*.” Researchers thus described varying viewpoints on research providing benefits to participants, but saw genuine benefits as an extension of designing for participant engagement in research.

4.2 Well-Being of Researchers

Researchers described significant impacts on their mental well-being associated with their focus on participant engagement, including surprise with how relationships with participants can influence decision-making and well-being. They felt establishing a professional distance is essential, but it can also be difficult for inexperienced researchers. Many voiced the importance of ensuring participants feel heard, with JR-P2 wondering if they were “*doing journalism or research*” when listening to participant stories and feeling obligated to publicize everything shared. JR-P6 described this direct engagement with participants as creating an “*unspoken social contract*” around participant experience sharing, which we understood as reflecting on the humanization of participants in qualitative research (i.e., in contrast to methods that remain more distant from participants and their experiences) that may create a feeling of obligation. JR-P5 candidly shared the emotional toll of health-related research, of “*[wishing they] knew that [research] was going to be so hard*,” the trauma of losing a participant unexpectedly, and their lack of preparation for such experiences.

Researchers studying emotionally charged topics like cancer, sexual abuse, and gun violence shared their surprise at how these impacted their well-being. Some expressed a desire to “*stay away from certain types of research... because [they] can’t seem to handle [what they might hear]*.” (JR-P6). Some researchers found interviews with participants in severe health contexts “*really exhausting*,” as some participants were eager to share while others struggled to engage (JR-P2). Some researchers were unprepared for hearing “*stories of trauma*,” but JR-P6 described

¹ Efficacy refers to the extent to which an intervention produces a beneficial effect under controlled conditions.

² Effectiveness pertains to the performance of an intervention in real-world settings.

noticing when participants “*opened up on heavy situations*” and then tried to refocus on the interview. However, pivoting like this was rare among junior researchers, who felt frustrated by emotional diversions and not knowing how to process them or their unrelatedness to the researcher’s expected research outcome. JR-P5 humorously advised researchers “*just be ready to spend two hours after every interview crying because you ... can’t really do anything for these people... health projects, [is] very taxing in a different way.*” Senior researcher SR-P4 shared a need for caution and planning, stating that research “*related to mental health care intervention is essentially a counseling intervention*” and that researchers should “[*be*] ... properly supported and guided” and “*have a plan for how to handle [participants] who didn’t respond the way you thought they would.*” They warned against researchers finding themselves acting as unqualified counselors, advising that researchers shut down interviews that become unmanageable and refer participants to appropriate services.

Researchers also reflected on related challenges in research groups and institutions. JR-P4 shared their struggle with a project member who suggested a participant had provoked the violence they suffered, effectively minimizing the participant’s experience. This had a lasting impact on JR-P4, highlighting junior researcher difficulties in emotionally charged topics when a project member asserted that a participant’s harm was attributed to something the participant must have done and was deserved. “*It’s been four years since [I had] that interaction, and that is [the] main thing [I] remember from that [project].*” Similarly, JR-P1 experienced tensions in a research group over goals around data cleaning. They felt members were “*trying to erase [participant] lived experiences*” because data did not align with intended findings and goals. Both JR-P1 and JR-P4 found these conflicts affected their mental well-being and soured them to certain research topics. Participants emphasized the value of research networks, especially for junior researchers who often rely on advisors and a research group. SR-P3 stressed that stronger communities might improve researcher well-being because “*stress and anxiety come from*” feelings of inadequate knowledge and that “*you might... already be the expert in that area... sometimes your mentor doesn’t even have [the] answer.*” SR-P3 highlighted the importance of diverse perspectives, noting that “*different kinds of answers*” emerge depending on that person’s role in research. A senior researcher will give advice different from that of a junior’s peer. JR-P7 expressed that their advisor, “*an assistant professor... [who] was very busy,*” provided only “*a little bit*” of training and, they “*mostly [learned] from other PhD students... but also just hands-on experience.*” JR-P2 expressed a desire for institutional support, saying “*universities should offer workshops to prepare students for this. I didn’t know I was going to go through all this, and training would help me manage it. It’s hard because [training] doesn’t bring money to the lab, but it’s an ethical responsibility for researchers.*”

5 Discussion

Researchers struggled to focus on increasing engagement due to a myriad of tensions. They perceived academic pressures prioritizing publishable data, while industry focused on product usability. Junior researchers felt unprepared for participant emotional distress, highlighting training needs. Some of their teams faced tensions in

balancing data integrity with participant experience. Researchers, in turn, suggested strategies to navigate these challenges.

5.1 Considerations for Researchers

How researchers perceive participant engagement influences the design and priorities of their studies. Academic researchers generally emphasized achieving precise and publishable results, whereas industry researchers focused on the success of their products. This distinction between probe-based and product-oriented approaches can influence researcher perceptions of engagement, ultimately shaping their strategies for effectively involving participants. For example, we heard that an emphasis on “clean” data may inadvertently lead researchers to exclude individuals with complex backgrounds, thus limiting valuable potential insights from the “edges” (SR-P4).

Senior researchers play a crucial role in mentoring junior researchers in Health HCI research, guiding them in fostering participant engagement and navigating the intricacies of publication. Moreover, they facilitate mentorship and networking opportunities, helping junior researchers recognize non-traditional success metrics. Senior researchers unanimously agreed that hands-on experience is the most effective way to learn research methods and identify suitable approaches. Asked what advice SR-P4 would give to other researchers navigating tensions regarding engagement, they replied they “*don’t think advice would help, [but experience will,] ... treat [all research outcomes] as a learning experience. Nothing is a failure.*” JR-P2 also emphasized that “*experience is a learning process*” and that it can be challenging to comprehend the situation and find solutions “*if you don’t actually experience this with participants through their data.*”

As students gain experience, they still rely on their advisors for guidance. SR-P3 noted the challenge of maintaining sensitivity to student needs, saying that “*Being reminded of what to discuss with students is valuable ... I appreciate guidance on how to be a better advisor.*” Senior researchers should try to consider issues from a junior researcher’s perspective and recognize their challenges. Researchers also suggested that obtaining validation from a wider network can help reduce feelings of imposter-syndrome, power hierarchies, and mental health challenges. Such factors can make it difficult for junior researchers to trust their instincts when addressing or following up on engagement opportunities.

Because advisors are not always available, SR-P5 recommended utilizing AI for exploring solutions and emphasized the importance of mentorship and community support in doctoral training. They provided several examples of how they used AI in their studies to engage participants effectively and also “*to help them brainstorm, compared to coming up with their own idea.*” By applying these insights, junior researchers can improve their skills, while senior researchers can adapt their mentoring for struggling students.

SR-P2 highlighted that traditional publication methods could hinder engagement in research, then suggested using social media for sharing “*shorter samples*” instead of waiting months for preprints and proposed hosting “*a lab meeting on Twitch*” to showcase projects. Strategies for aligning research with participant benefits include early participant engagement, adaptable research designs, partnerships with industry, iterative prototyping

and feedback, and maintaining participant involvement in the research process.

SR-P1 highlighted a need for transparency with participants, stating it is essential to communicate the workload of research participation, noting *“independent of the compensation ... there is a lot of work.”* Although emphasizing the effort needed to participate might deter some potential participants, SR-P1 confirmed many of their participants still expressed interest in the study after learning about the required engagement. SR-P3 emphasized the significance of building rapport, referring to it as *“invisible work”* while noting that long-term engagement is not easily incentivized, nor can rapport be easily scaled. They stressed that meaningful engagement brings valuable insights, which cannot be rushed or replaced by technology, underscoring a need for careful planning and adaptation.

5.2 Considerations for Institutions

Many researchers described that academic researchers prioritize publication, creating an environment not conducive to exploring a lack of participant engagement. They suggested that introducing alternative incentives and recognizing a breadth of impact and dissemination methods for research could greatly benefit the field. This change would also align with research on translation gaps in Health HCI, which notes that many practitioners find papers cumbersome to engage with [2]. Such a change might also help reveal valuable insights and solutions currently overlooked due to limited publication opportunities. Based on the experiences shared by researchers, some have opted for study designs and methods that prioritize obtaining clean data and securing publication rather than exploring engagement issues related to their chosen topics.

Institutions could offer specialized training programs focused on developing skills in human-centered design, communication, and tailored recruitment techniques for diverse participants. Programs like Stanford Medicine’s Participant Engagement Consultations [15] and Research Participant Engagement Program [14], and the University of Minnesota’s have Recruitment Support [13] provide no-cost assistance for researchers throughout the participant engagement process, including study design and outreach. Stanford’s program included assistance reaching potential participants through an “honest broker” and connecting with co-investigators at other institutions [15]. Additionally, Institutions should consider providing training in community-based participatory research (CBPR), which has been shown to enhance recruitment and retention of diverse populations by involving community members in the research process and forming partnerships with community-based organizations [17]. CBPR can facilitate quicker translation of research findings into community action and make them more accessible to healthcare providers. These and other trainings would benefit researchers lacking experience with diverse populations, possibly benefiting populations that are difficult to recruit or maintain engagement with, as researchers described avoiding related research topics or not meaningfully exploring engagement strategies within their ongoing studies.

Additionally, establishing robust mentorship programs connecting early-career researchers to more experienced

professionals can facilitate knowledge sharing and support throughout research. Such mentorship can include guidance on managing challenges related to participant engagement and fostering resilience in the face of difficulties. Institutions like Cornell [3] and UCLA [16] have grants for engagement research. Institutions could create Collaborative Research Networks (CRNs) [1] to encourage interdisciplinary partnerships, allowing researchers to learn from different fields and incorporate innovative engagement practices [1, 6, 10]. SR-P3 advocates for increased mentoring opportunities within the academic community, stressing the importance of checking in and normalizing feelings of uncertainty among peers. By integrating such supports, institutions can cultivate an environment prioritizing participant engagement while empowering researchers to produce impactful and inclusive research.

5.3 Considerations for the Field

Researchers emphasized the importance of early engagement and consistent communication in fostering a sense of trust and encouraging ongoing participation in studies. Researchers felt it was crucial to communicate the expected level of effort involved in participation and to integrate participants into decision-making processes. Others felt that offering training on the research process would be beneficial, as such training could equip researchers with the knowledge to contribute meaningfully to the field while motivating researcher attention to participation and engagement. Ensuring that researchers feel confident conducting research on understanding and addressing barriers that prevent participants from engaging would help foster a more significant commitment to this research. Such a multifaceted approach can help improve participation in Health HCI research and catalyze deeper exploration of strategies to increase engagement in research activities.

Researchers felt that human-centered design culture within research teams could thrive on actively seeking input for continuous improvement, including by organizing workshops that could further promote collaborative ideation between researchers to enhance participant engagement. Targeted workshops focusing on communication and motivation strategies were seen as beneficial, as were seminars on human-centered design principles. Many researchers advocated for peer mentorship programs and encouraged collaboration. They also felt that informal networking events facilitate the sharing of challenges and solutions, thus reinforcing a culture of shared learning, and that a centralized online resource center could offer self-paced learning materials. Finally, structured feedback sessions could allow researchers to refine their engagement strategies for greater effectiveness.

6 Conclusion

Our investigation into the perceptions of Health HCI researchers regarding participant engagement has uncovered important insights into the challenges and disparities experienced by academic and industry researchers. Findings suggest that the prevalent “publish or perish” mentality in academia often overshadows the need for meaningful connections with participants and thus hinders efforts to improve participation and engagement. On the other

hand, industry researchers were perceived as more focused on practical applications and success, creating different incentives that can enhance participant engagement.

The emotional and ethical complexities surrounding participant engagement pose additional hurdles, particularly for junior researchers who may feel ill-equipped to address such challenges. The need for targeted training and support emerges as a crucial recommendation, emphasizing the importance of equipping researchers with the tools and strategies necessary to deepen participant connections and improve health outcomes.

This research ultimately underscores the necessity of re-evaluating incentive structures within the Health HCI research landscape. By bridging gaps between academic and industry approaches and prioritizing participant engagement, researchers can enhance the relevance and quality of their contributions to health technology, fostering a more impactful dialogue between technology and healthcare. Moving forward, a concerted effort to integrate best practices in participant engagement across all sectors of Health HCI research will be essential for advancing the field and improving overall participant outcomes.

A limitation of this study is the limited access to a diverse pool of researchers for interview purposes. Due to resource constraints and geographical limitations, study researchers may not fully represent the range of experiences and perspectives in Health HCI. This limitation could have implications for the generalizability of the findings, and our results also should not be considered comprehensive as a lack of access to specific groups or individuals may have introduced bias in data collection. Future research should address these limitations by exploring researcher experiences and perspectives around participation and engagement in Health HCI.

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